

Policy Brief: Chronic Sleep Deprivation Among Informal Caregivers: Implications for Burnout, Long-Term Health Risks, and Health System Capacity

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Policy Question: *How does chronic sleep deprivation among carers of medically complex and ageing individuals contribute to caregiver burnout and long-term health risks, and what role should targeted funding for respite and sleep-protective supports play in sustaining care quality and health system capacity*

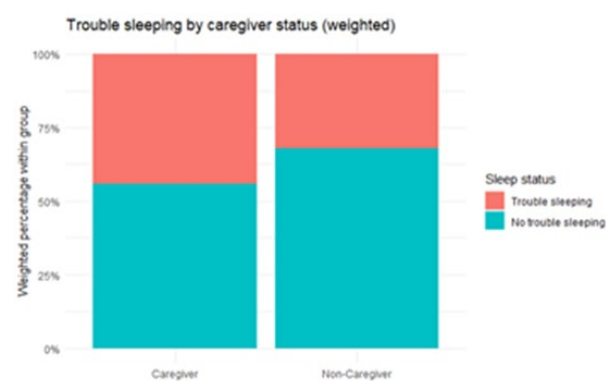
Key tensions: Availability of respite services; Inconsistent access to caregiver supports; Social and health policy responsibility

Problem definition

Caregivers frequently experience chronic sleep deprivation due to the intensity and continuity of care responsibilities, particularly when caring for individuals with advanced age, disability, or medical complexity. 1 in 4 Canadians currently provides care to a loved one with a medical condition or challenges related to aging (Statistics Canada 2022a). This includes overnight monitoring, medication management, or responding to emergencies. The degree of sleep disruption increases as care needs intensify. This reliance on informal caregiving carries significant economic implications for the broader health system. Canadian caregivers provide an estimated 5.7 billion hours of unpaid care annually, with an economic value of approximately \$97 billion that is not reflected in conventional health budgets (Fast et al., 2023). 1 in 4 Canadians currently provides care to a loved one with a medical condition or challenges related to aging (Statistics Canada 2022a). Demand for informal caregiving is expected to grow further as Canada's population aged 85 and older is projected to triple by 2046 (Statistics Canada, 2022b) and as formal care systems face workforce shortages and capacity constraints. Despite this growing dependence on unpaid caregivers, access to social and practical supports remains limited and inconsistent across regions.

As a result, caregivers face heightened risks of burnout, chronic illness, and poor mental health. These outcomes not only affect caregiver wellbeing but also increase health care utilization and the likelihood of care breakdown, placing additional pressure on already strained health systems. As caregiver health declines, those being cared for and caregivers themselves use more health services, meaning health systems reabsorb not only the savings of unpaid care, but also the downstream health and economic consequences of caregiver strain.

Policy Context and Evidence



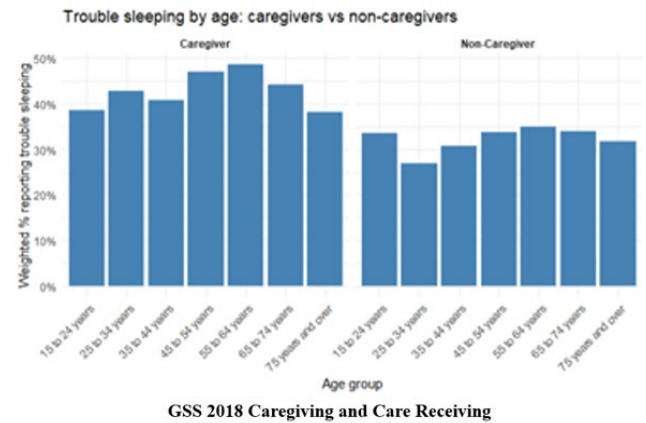
Poor sleep quality is increasingly recognized as a population health concern because it is associated with adverse physical and mental health outcomes (Li et al., 2022). PHAC-published research notes that poor sleep is linked with outcomes such as poorer self-rated health, obesity, cardiovascular disease, and higher all-cause mortality risk, and it also examines connections between sleep duration and quality and indicators of mental health, including mood and anxiety disorders and suicidal ideation (Clayborne et al., 2023). These risks matter for caregivers because sleep disruption is not an isolated symptom.

As caregiving demands increase, especially when care requires overnight monitoring and complex tasks, sleep disruption becomes a key pathway to burnout, deteriorating health, and eventual care breakdown. Canadian survey evidence

shows a clear link between caregiving, sleep disruption, and caregiver distress. Data from the 2018 General Social Survey (Caregiving and Care Receiving), caregivers were more likely than non-caregivers to report trouble

sleeping (44.1% vs. 32.0%). This sleep burden coincides with broader caregiver strain. CIHI reports that more than one in three unpaid caregivers of home care clients experience distress, including depression and difficulty sustaining caregiving responsibilities, placing continuity of home and community care at risk. Across Canada, access to caregiver supports remains fragmented and uneven, with substantial variation in the availability and quality of respite and practical supports across provinces and territories

Canadian Centre for Caregiving Excellence [CCCE], 2022). Services for caregivers and care recipients are often insufficient, difficult to access, and poorly aligned with the needs of employed caregivers, particularly in relation to time off and recovery from caregiving demands. Caregivers report losing significant time and sleep not only to direct care but also to navigating complex service systems, and very few have access to formal support that provides predictable relief from caregiving responsibilities. This is particularly relevant for respite where caregivers report not getting access (CCCE 2022). However, targeted pilots such as SE Health's Respite for All program demonstrate that work-protective, home-based respite for employed caregivers, integrated with mental health supports, however notably not capturing impacts on sleep (Toronto Central Health Line, n.d). Programs like this highlight potential for feasibly delivered through coordinated partnerships. These disparities underscore the absence of consistent national expectations and infrastructure for respite and sleep-protective supports (CCCE, 2022). Internationally, the United Kingdom provides a relevant policy comparator by framing caregiver support as both a health and labour issue. There are approximately five million working carers in the UK, supported by national workplace protections including The Flexible Working Act, securing rights and job protection for caregivers (Carers UK, n.d.). In 2024, the UK introduced a statutory entitlement to up to one week of unpaid carer's leave, establishing a national baseline for caregiver support. This experience demonstrates the feasibility of pairing respite expansion with employment protections to reduce caregiver strain and support the sustainability of informal care (Carers UK, n.d.).



Policy Options

Option 1: Maintain the Status Quo

Provincial and territorial governments would make no significant changes to existing respite or caregiver support programs. Informal caregivers would continue to seek out respite when available, working with social workers and medical staff to get support where needed. This option carries the lowest immediate fiscal cost but the highest long term system risk.

Option 2: Expand Respite Availability

This option focuses on provinces and territories expanding respite as a core continuing-care service, increasing access to short term and overnight respite services through both community and facility-based models. Measures would include, more home care nurses, integrating dedicated respite capacity within long term care facilities through rotating or designated respite units. Improved respite access would allow caregivers to rest and recover, reducing the likelihood of caregivers reporting worse sleep given the increased rest time. By reducing cumulative fatigue and burnout, targeted respite funding plays a critical role in sustaining informal care capacity, delaying more intensive service use, and supporting the long-term quality and stability of care across the health system.

Option 3: Introduce Caregiver Sleep Protection Supports

This option reframes caregiver sleep deprivation as both a health and labour policy issue, recognizing that many caregivers simultaneously sustain paid employment and intensive care responsibilities. Measures could include paid caregiver days off work, strengthened flexible employment protections, and a government-funded caregiver sleep protection allowance. By targeting workplace conditions alongside care supports, this option acknowledges the multifaceted role of caregivers and the interdependence of paid work and unpaid care.

Recommendation

This brief recommends a coordinated strategy to reduce caregiver sleep deprivation and burnout by combining targeted respite expansion with system-level supports for high-risk caregivers bringing together key elements from option 2 and 3. Governments should pilot a bi-monthly weekend respite program, stratified by care intensity (mild, moderate, and severe), to prioritize caregivers managing medically complex or high-intensity care demands and to evaluate the impact of reliable respite on sleep burden and fatigue. In parallel, new long-term care developments should standardize the inclusion of rotating respite wings, embedding short-term and overnight respite capacity directly within existing LTC infrastructure and workforce skillsets rather than creating parallel service streams. To reinforce these service-based supports, a provincial framework should be introduced to evaluate and incentivize care-friendly workplace policies, such as paid caregiver leave and flexible scheduling, that protect caregiver sleep, sustain labour force participation, and stabilize informal care capacity. Together, these measures offer a scalable, evidence-generating approach to supporting caregiver health while strengthening long-term care system resilience.

Implementation and next steps

- **Targeted Respite Pilots Anchored in Sleep Protection:** These pilots would provide guaranteed overnight or weekend respite, giving caregivers protected opportunities for uninterrupted sleep. Access would be guided by standardized assessments of care intensity and sleep disruption, ensuring that those experiencing the most severe and sustained sleep loss are prioritized. Data collected before and after respite use would focus on sleep quality, fatigue, and caregiver functioning, building early evidence on health and system impacts.
 - **Next Step** Use a standardized clinical tool (e.g., the Zarit Burden Interview or a sleep-specific burden scale) to classify caregivers into intensity tiers, with participants reporting sleep quality and fatigue pre- and post-respite to assess impacts on caregiver health and potential health-system savings.
- **Embedding Sleep-Protective Respite Within Care Infrastructure:** Sleep protection must also be built into care delivery systems. Long term care and community care infrastructure should include dedicated short stay or rotating respite capacity that supports overnight relief as a routine service rather than an emergency measure. These units would be staffed through existing clinical teams, ensuring continuity of care, while centralized scheduling would allow caregivers to plan rest periods in advance and access rapid support when sleep deprivation reaches crisis levels.
 - **Next step** Provincial mandate new long-term care builds to include designated rotating respite units staffed by existing clinical teams, with beds managed through a centralized booking system that supports both planned overnight relief and rapid emergency access when caregiver sleep deprivation reaches crisis levels.
- **Enabling Access Through Workplace and Financial Supports:** Sleep-protective care must be supported by aligned labour and income policies. Paid caregiver days, flexible scheduling protections, and a care-friendly employer designation, supported by targeted tax incentives, would allow caregivers to make use of respite services without risking employment or financial stability.
 - **Next Step** Establish a “Care-Friendly” employer seal tied to paid caregiver leave and flexible scheduling, support uptake through tiered tax incentives for Small and Medium-Sized Enterprises, and monitor employee retention and health outcomes to assess impacts on workforce stability and caregiver sleep and well-being.

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